

**SHRI BALAJI KIDNEY CARE**

PVT. LTD

Visit No:	20260121/1	Age / Sex:	24 / Male
Patient Name:	MAULIK GHANSHYAMBHAI THUMAR	Collected Date:	21-01-2026
Referred By :	DR DHANANJAY	Received Date:	21-01-2026
Referred From :	CONTINENTAL HOSPITAL	Reported Date:	22-01-2026
Specimen:	Tissue		

Allograft Renal biopsy LM+IF+C4D

Specimen: Allograft Kidney (needle) biopsy

H26- 90

Brief Clinical History:

24 years old male, CGN-CKD, s/p live related Tx, k/c/o live renal Tx recipient 7/1/26, mother donor, HCA - 3/6, CDC/flow/DSA-Negative, Tx done with ATG induction, currently on triple immunosuppression, Tac/MMF/steroids, Allograft dysfunction - increasing creatinine, serum creatinine : 1.25 to 1.58 to 1.74mg/dl, CUE : Protein : Trace, RBC : 15 to 20/HPF, CBC : Hb : 10, WBC : 17640, Platelets : 487000, Albumin : 3.9g/dl, LFT : Normal, Tacrolimus : 2.6 to 4.11, Acute allograft dysfunction, ?Rejection.

Specimen: Allograft Kidney (needle) biopsy**Gross Pathology:**

1. Received , three linear gray white cores, in 10% NBF, measuring 1cm, 0.4cm and 0.3cm with fat in length. All tissue processed for light microscopy.
2. Received, single linear gray white core, in Michel's media, measuring 0.9cm in length. All tissue processed for Immunofluorescence study.

Microscopic Description:

Serial sections and stains (H&E, PAS, Methanamine Silver & Masson Trichrome) studied show 03 cores; 02 fibrofatty tissue and other third core of renal subcapsular cortex with adjoining part of medulla with up to 08 glomeruli and an artery.

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Glomeruli:

A single glomerulus is obsolescent. The viable glomeruli appear unremarkable with no evidence of significant increase in cellularity ,glomerulitis, or fibrin thrombi.

Tubules:

There is focal mild tubular injury with few tubules (about 30%) showing loss of the brush border. Occasional tubules show evidence of mild mononuclear tubulitis seen in the biopsy studied. A rare tubule shows evidence of neutrophilic* cast. There is no evidence of significant tubular atrophy/interstitial fibrosis seen in the biopsy studied.

Interstitium:

The interstitium shows a mild to moderately dense patchy infiltrate of lymphocytes ,occasional plasma cells and rare neutrophils* seen scattered and in an aggregates at two places; involving about 20% to 25% of the cortex sampled.

Vessels:

There is evidence of moderate to occasional marked peritubular capillaritis involving about 10% to 15% of the peritubular capillaries. Mild arteriolar hyalinosis is seen .There is no evidence of arteriosclerosis/endarteritis seen in the biopsy studied.

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Immunofluorescence:

IF study shows 02 glomeruli with no immune deposits.

Immunohistochemistry:

IHC for C4d stain does not show positivity around the peritubular capillaries .

Diagnosis/Comments:

Allograft Kidney (needle) biopsy shows features of mild interstitial inflammation with mild mononuclear tubulitis consistent with Borderline changes suspicion of an Acute T-cell mediated Rejection.

*In view of rare neutrophils seen in the interstitium and in a rare tubule, a remote possibility of an associated ascending infection cannot completely be ruled out. Advised urine culture and sensitivity test to rule out and confirm the same.

In view of marked peritubular capillaritis, a remote possibility of an concurrent probable active antibody mediated rejection cannot completely be ruled out. Advised serum DSA levels to confirm the same.

Banff score :g0, t1, i1, v0, cg0, ct0, ci0, cv0, ptc3, C4d 0, mm0, aah1, ti1, i-IFTA 0.

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	No.	IgG	IgM	IgA	C3c	C1q	Kappa Lc	Lambda Lc
I.Glomeruli	02							
Localization		-	-	-	-	-	-	-
Pattern		-	-	-	-	-	-	-
Intensity		-	-	-	-	-	-	-
II.Tubules		-	-	-	-	-	-	-
Intensity		-	-	-	-	-	-	-
III.Vessels		-	-	-	-	-	-	-
Intensity		-	-	-	-	-	-	-

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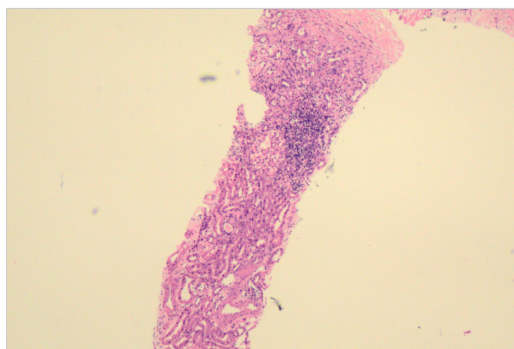
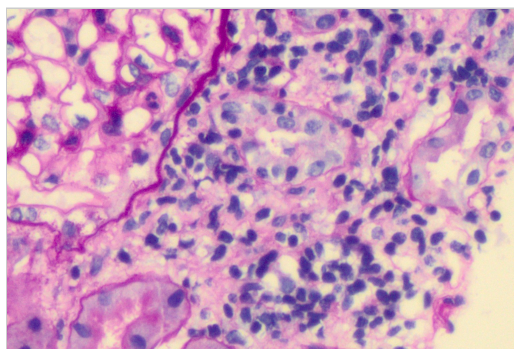
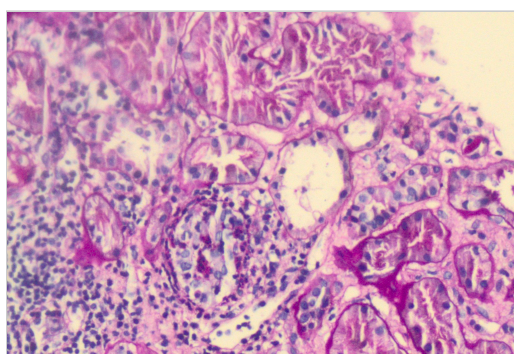
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